INFORMATION TO KEEP WITH YOU

# My complete treatment handbook

## Weekly cetuximab

Understand your treatment. Know what to expect. Keep your plan to hand.

**Displayed calendar**
**7 days**
See the active treatment phase

**Treatment stages**
**1**
Explained in the treatment reference

**Regimen source**
**NCCP 00207**
Version 6

> One larger first infusion, then cetuximab once every 7 days while treatment is helping and side effects remain acceptable. This is cetuximab alone, with no home chemotherapy pump.

### Treatment stages

01 **A 7-day reference cycle**
Clinic → home → review

Your team confirms the active stage and dates. The full treatment guide explains each step.

### Inside your handbook


| | | PAGE |
|---|---|---|
| 01 | Understanding your treatment | 2 |
| 02 | Your schedule, tests and appointments | 5 |
| 03 | Symptoms and supportive care | 10 |
| 04 | Your record and contact card | 17 |
| 05 | Sources and further information | 20 |


**My name** _____

**Hospital / oncology team** _____

> Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

# Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

EGFR-expressing RAS wild-type metastatic colorectal cancer after the specified previous therapies

## When to get help

**Emergency: call 112 / 999**

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

**Contact your oncology service immediately**

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. New breathing difficulty, chest pain, collapse, severe diarrhoea or inability to drink require prompt assessment. Report eye pain, blurred vision, blistering or infected skin promptly.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

## How your medicines work

To control EGFR-expressing RAS wild-type bowel cancer that has spread after oxaliplatin- and irinotecan-based treatments have failed and irinotecan cannot be tolerated.

EGFR-TARGETED ANTIBODY

### Cetuximab

Erbitux



**Blocks growth messages at the cell surface**

Cetuximab attaches to EGFR on the outside of cells and blocks growth messages. In bowel cancer, a validated RAS wild-type result helps identify whether this approach is suitable. Healthy skin also uses EGFR, so skin and nail effects need early care.

EGFR receives a growth signal → Cetuximab blocks the receptor → Some growth signals are reduced

**What to watch for**

Report rash or sore nails early. Tell the nurse immediately about infusion symptoms. Eye pain, new breathing problems or severe diarrhoea need prompt advice. Blood tests check minerals during treatment and for two months afterwards.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

## Your treatment aim

The setting changes what benefit means.

**Why this option?**

Cetuximab alone is an option for RAS wild-type bowel cancer that has spread after oxaliplatin and irinotecan have failed, when irinotecan cannot be tolerated. Your team explains the aim, alternatives and how benefit will be reviewed.

**A plan that can change**

For bowel cancer, discuss control of cancer and symptoms, and whether further local treatment may become possible. Your preferences and priorities matter.

## Why this treatment is offered

In selected previously treated bowel cancer, cetuximab improved disease control and survival compared with supportive care.

CHAPTER 02

# Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

# Treatment, recovery and review

**Weekly cetuximab**

7-day view




This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

CLINIC → HOME → REVIEW

## A 7-day reference cycle

D1 D7

Cetuximab on Day 1. No home chemotherapy pump. A repeating 7-day cycle. The number of cycles depends on benefit, side effects and your treatment plan.

This is a general explanation, not an appointment calendar. Delays or changes are decided by your team. Each medicine can need a different change.

BASELINE → TREATMENT → FOLLOW-UP

## Treatment checks across the course

Relative stages, not equally spaced dates. Agree who books each check, reviews results and gives advice if a check is missed.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00207 v6 · official regimen

## Tests in your plan

### Carcinoembryonic antigen (CEA) — colorectal plans

If part of your plan

WHEN

**For a colorectal cancer plan, if your team uses CEA; your team sets the interval.**

A marker trend interpreted alongside symptoms, examination and scans.

Record the baseline, latest result and next date. A marker change alone does not establish progression.

THE WHOLE TREATMENT JOURNEY

## Weekly cetuximab: treatment and checks

**Selected metastatic colorectal cancer; or locally advanced head/neck cancer with radiotherapy when platinum is unsuitable**

One larger first infusion is followed by weekly infusions. Bowel-cancer treatment continues while helping and tolerated. For head-and-neck treatment, cetuximab starts one week before radiotherapy and continues until radiation treatment finishes.

**Read from left to right.** Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

**Treatment, tests and reviews across the course**

| Care / check | Before starting | First treatment | During treatment | Phase / dose review | After treatment |
|---|---|---|---|---|---|
| **Bloods and symptoms** NCCP 00207 | Baseline blood count, kidney and liver checks | Confirm tests and prescription before the first dose | Repeat bloods and symptom checks according to this regimen | Recheck after a hold or treatment change | Follow unresolved effects and the agreed disease-review plan |
| **Treatment review** NCCP 00207 | Confirm exact cancer setting and biomarkers | Explain expected benefit and how it will be assessed | Report new symptoms between visits | Review response, tolerability and active medicines | Agree the next treatment or surveillance plan |
| **Electrolytes and EGFR effects** NCCP 00207 | Check electrolyte and skin-risk plan | Skin care and prompt reaction reporting | Monitor magnesium, calcium, potassium, skin and eyes | Do not stop follow-up because the infusion stops | Monthly electrolytes, magnesium and calcium for two months after the last EGFR-antibody dose |

## What each check is for

### 1 A 7-day reference cycle

Clinic → home → review

For selected bowel cancer that has spread, cetuximab aims to control the cancer after previous treatments. For selected locally advanced head-and-neck cancer when platinum chemotherapy is unsuitable, it is combined with radiotherapy. Your team confirms which pathway applies.

**Ask / act:** Confirm the current phase, prescription, tests and support medicines.

## At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

## Infusion visits and care between treatments

Cetuximab on Day 1. No home chemotherapy pump. A repeating 7-day cycle. The number of cycles depends on benefit, side effects and your treatment plan.

STEP 1

### Treatment phases and appointments

STEP 2

### Checks after treatment

Monthly electrolytes, magnesium and calcium for two months after the last cetuximab dose; assign responsibility before treatment ends.

STEP 3

### Access and practical support

Day-unit intravenous (IV) access; no home fluorouracil pump.

#### Do I need a port?

A peripheral cannula or existing central line may be used. A port is not automatically needed; the team considers your veins and overall treatment plan.

#### If the schedule changes

Ask the unit to confirm the next visit and which medicine is being changed. Do not copy dates from another regimen.

#### Observation at every antibody visit

Premedicines and monitoring add time to every cetuximab visit. Your team will confirm the observation period before you leave; do not assume that later visits can omit observation.

## Before the first treatment

Checks reduce avoidable risk.

### Blood tests and medicines

Review blood counts, kidney/liver tests, symptoms, allergies and all medicines and supplements. Tell the team about red-meat allergy or tick bites before cetuximab because these can be associated with infusion-reaction risk.

### Preparing access

Allow for blood tests, review, pharmacy preparation, infusion and observation. A port is not automatically required; ask which access is suitable.

## Visits and treatment sequence

Know which steps happen in the clinic and at home.

## When treatment changes or ends

Follow-up continues.

### Symptoms and monitoring

Continue the cetuximab skin plan and mineral checks for two months after the last dose. Ask which medicines to continue, when to seek help and how response will be assessed.

### Next decisions

The team reviews benefit, side effects and your priorities. Supportive and palliative care can help alongside active treatment. Ask for a clear next-step and contact plan.

CHAPTER 03

# Symptoms and supportive care

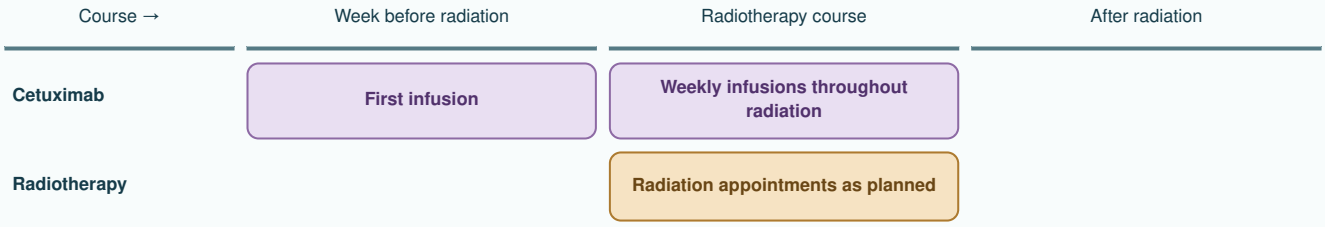
IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

THE WHOLE TREATMENT COURSE

## Head-and-neck indication: cetuximab before and during radiotherapy

This panel applies only to the head-and-neck radiotherapy indication of NCCP 00207. Cetuximab is a targeted antibody. Colorectal cetuximab alone has no routine radiotherapy component.




The axis shows order, not equal durations. Cetuximab starts one week before radiation and continues until radiation ends. The radiation team sets the number of sessions and confirms eligibility.

> **After the course: continue skin, mouth, swallowing and nutrition support. Mineral monitoring continues for two months after the final cetuximab dose.**

Educational reference, not a personal appointment calendar. NCCP 00207 v5, page 1; current EMA Erbitux information

## Day 1 → Day 7: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

One larger first infusion, then cetuximab once every 7 days while treatment is helping and side effects remain acceptable. This is cetuximab alone, with no home chemotherapy pump.

| Treatment or check | Timing in this view |
|---|---|
| **Cetuximab infusion** |  **Day 1** intravenous (IV) |
| **Skin, nails and minerals** |  **Days 1–7** Can build over successive cycles |
| **Energy and bowel symptoms** |  **Days 1–7** Variable timing |

**Your planned visits:** Cetuximab on Day 1. No home chemotherapy pump. A repeating 7-day cycle. The number of cycles depends on benefit, side effects and your treatment plan.

**Care continues:** Continue the skin-care plan and mineral checks arranged after cetuximab. Ask which medicines to continue and when your next review is planned.

**No guaranteed safe days:** An infection, reaction or other urgent problem can occur at any time. The graph does not predict a personal low point.

**Why these bands stay broad:** NCCP defines the treatment days. Symptom bands are broad educational guidance; reactions can occur on any day, and cumulative effects may continue between cycles. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00207 v6 · official regimen ↗

**There are no guaranteed "safe days".** Fever, shivers or suddenly feeling unwell require immediate contact. Serious symptoms override any timeline or diary.

### Between-treatment notes

### Day 1: Treatment and checks

The team reviews blood tests, symptoms and your prescription before treatment. Premedicines and observation are part of cetuximab visits. Know the contact number and the next planned visit.

### Day 2–3: Care between visits

Follow the skin-care or bowel-symptom plan provided. There is no pump to remove. Call early for symptoms; do not wait for the next appointment.

### Day 4–7: Recovery and the next review

The team uses symptoms, tests and your priorities to decide what happens next. Bring questions and a symptom record if useful; this is optional.

## Support around the medicines

### 1 First cetuximab dose

Skin care and sun protection under local EGFR policy. Premedicines and monitoring add time to every cetuximab visit. Your team will confirm the observation period before you leave; do not assume that later visits can omit observation. Report delayed reaction symptoms urgently. Monthly electrolytes/Mg/Ca for two months after last dose.

### 2 Subsequent weekly cetuximab

Premedication continues for subsequent infusions. Post-treatment monthly electrolytes/Mg/Ca for two months.

### 3 Previous infusion reactions

Tell the team about any previous severe antibody reaction, even if you feel well now. Premedication does not remove all reaction risk. Report breathing problems, swelling, faintness or rapidly worsening symptoms immediately.

## Symptoms in context

Report what changed and how it affects you.

### Bowel symptoms

Follow your written rescue plan. Fever, blood, severe pain, reduced urine or inability to drink needs urgent advice.

### Skin and nail changes

Start the prescribed skin-care plan early, use moisturiser and sun protection, and report painful or infected skin or nails. Keep mineral blood-test appointments during treatment and for two months after cetuximab.

## Living with treatment

Use the support available.

### Food, activity and practical help

Use prescribed anti-sickness medicines, pace activities and ask for nutritional, emotional or practical support.

### Pregnancy and fertility

Discuss fertility preservation and reproductive plans before treatment. Ask for the contraception and breastfeeding instructions for every medicine; the whole regimen may require longer precautions than one medicine alone.

## Questions worth asking

**Do I need to enter dates to understand the cycle?**

No. The graph explains a general pattern. Your hospital arranges appointments; the guide and its safety information are available without entering anything.

**Will I have a pump at home?**

There is no home fluorouracil pump in this regimen. Your team chooses a cannula or central line based on your veins and wider treatment plan.

**What happens if a dose is delayed?**

The team checks the cause and may delay, reduce, change or stop one medicine. Ask which component changes and when you should next attend; do not restart yourself.

**How will I know whether it is helping?**

Scans, symptoms and clinical review assess benefit. A rash, side effect or good day alone cannot show whether the cancer is responding.

The graphic below shows the relationship.

### Why is RAS testing mentioned?

RAS testing selects cetuximab treatment in colorectal cancer. Confirm the cancer-specific plan with your team.

> **Try explaining the plan back:** Can you explain which visits are planned, whether a pump is involved, and which symptoms mean you should call today? No dates or symptom scores are needed to use this guide.

---

**Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

## First cetuximab dose

**Active medicines:** cetuximab

**Emetogenic risk:** low

| Medicine | Dose / route | When |
|---|---|---|
| Chlorphenamine | 10 mg intravenous (IV) | 60 minutes before cetuximab<br>NCCP suggested premedication. |
| Dexamethasone | 8 mg IV | 60 minutes before cetuximab<br>NCCP suggested mandatory infusion premedication; reconcile low-risk antiemetic support to avoid duplicate steroid. |
| Rescue antiemetic | As individually prescribed | If required<br>Low emetic risk; use current NCCP/local guidance and account for the infusion steroid. |

### Additional phase-specific support

- Skin care and sun protection under local EGFR policy.
- First two doses require post-infusion observation; delayed-reaction education.
- Monthly electrolytes/Mg/Ca for two months after last dose.
- NCCP p1 permits cessation of observation after two tolerated cetuximab doses, whereas Erbitux SmPC section 4.2 requires monitoring during and for at least one hour after each infusion. Reconcile and document the local observation plan; do not promise early discharge.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

---

## Subsequent weekly cetuximab

**Active medicines:** cetuximab

**Emetogenic risk:** low

| Medicine | Dose / route | When |
|---|---|---|
| Chlorphenamine | 10 mg intravenous (IV) | 60 minutes before each infusion |
| Dexamethasone | 8 mg IV | 60 minutes before each infusion<br>Reconcile antiemetic steroid; no automatic duplication. |

### Additional phase-specific support

- Premedication continues for subsequent infusions.
- Post-treatment monthly electrolytes/Mg/Ca for two months.
- NCCP p1 permits cessation of observation after two tolerated cetuximab doses, whereas Erbitux SmPC section 4.2 requires monitoring during and for at least one hour after each infusion. Reconcile and document the local observation plan; do not promise early discharge.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

# What to watch for

You do not need to know which medicine caused a symptom before calling.

### 1 Eyes

**Cetuximab**

Eye pain or new visual change needs prompt advice.

### 2 Blood minerals

**Cetuximab**

Tests check magnesium and other minerals even when you feel well.

### 3 Breathing / reaction

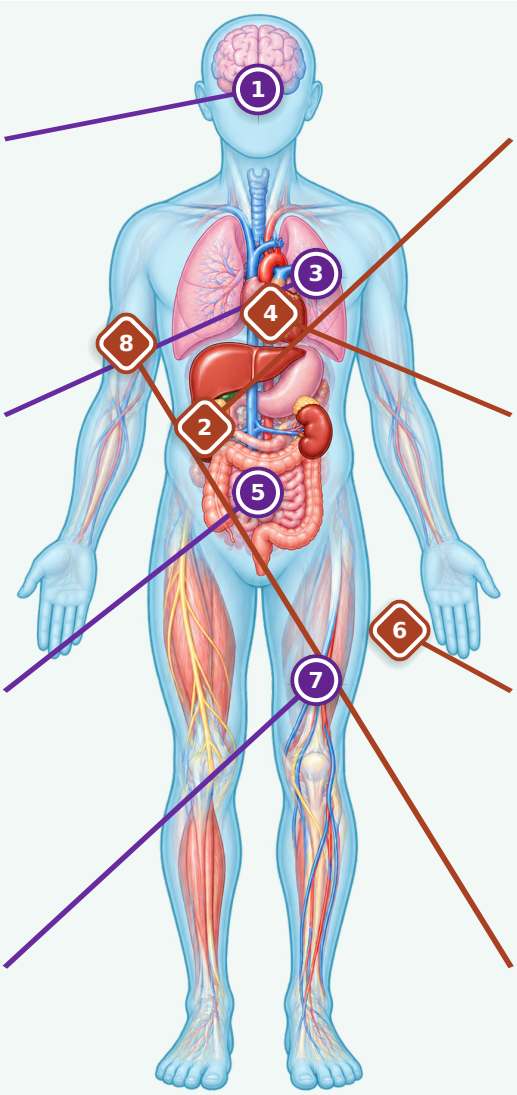
**Cetuximab**

New breathing difficulty requires prompt assessment.

### 4 Infusion symptoms

**Cetuximab**

Tell the nurse immediately about itching, swelling or flushing.




### 5 Bowel

**Treatment**

Diarrhoea, increased stoma output or dehydration: call early.

### 6 Skin / nails

**Cetuximab**

Report painful rash, cracks or infected skin early.

### 7 Blood / infection

**Treatment**

Fever, shivers or sudden illness: contact oncology immediately.

### 8 Infusion access

**Treatment delivery**

Pain, swelling, leakage or a loose connection needs immediate advice.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

New breathing difficulty, chest pain, collapse, severe diarrhoea or inability to drink require prompt assessment. Report eye pain, blurred vision, blistering or infected skin promptly.

**When to get help**

**Call 112 / 999** for severe breathing difficulty, severe chest pain, collapse or a seizure.

**Call oncology immediately:** temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

**Oncology daytime:** 087 12345678 · **Out of hours:** 087 87654321

**EXAMPLE ONLY — team must replace both numbers before patient use.**

CHAPTER 04

# Your record and contact card

IN THIS CHAPTER

Space for your own cycle or review record, questions and notes, followed by your treatment card.

Record each cycle separately. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Use a new record sheet for each cycle or review. Copy or print another sheet when needed; the number of sheets does not prescribe the length of treatment.

Cycle number: _____ Start date: _____ Next review: _____

## Cetuximab · EGFR-targeted antibody

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Skin / nail changes** | □ None | □ Mild change: report | □ Pain or daily tasks affected: call | □ Spreading infection or blistering: urgent review | □ Severe illness / collapse: emergency help |
| **Eye symptoms** | □ None | □ New irritation: report | □ Pain, redness or light sensitivity: call promptly | □ Blurred vision / severe eye pain: urgent review | □ Sudden major vision loss: emergency assessment |
| **Diarrhoea** | □ Usual pattern | □ Up to 3 extra stools above normal | □ 4–6 extra stools/ day or at night: call | □ 7+ extra stools/ day or dehydration: urgent review | □ Collapse / severe dehydration: 112/999 |

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

# Keep your treatment details to hand

**EGFR-expressing RAS wild-type metastatic colorectal cancer after the specified previous therapies** · NCCP 00207 v6 · Document revised 7 November 2026

Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

**FRONT / CONTACTS**

## My Treatment Card

SACTCheck

Name: _____

**Weekly cetuximab — bowel cancer**

Cetuximab once every 7 days; ongoing treatment review.

Phase: _____ Cycle plan: team to confirm

**Daytime (EXAMPLE)** 087 12345678

**Out of hours (EXAMPLE)** 087 87654321

**Unwell or fever? Call oncology now.**
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

CUT outer border

**BACK / TREATMENT RECORD**

## My cycle record

Rolling record; the team confirms the total treatment plan.

| Cycle | Date / initials |
|---|---|
| | |
| | |
| | |
| | |

My treatment information

Latest treatment date: _____

Plan changed? Ask the team to update this card.

**FOLD centre — do not cut**

## Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

## Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

### iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

### Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

## Your local contacts and resources

**Oncology daytime:** 087 12345678 · **Out of hours:** 087 87654321

**EXAMPLE ONLY — team must replace both numbers before patient use.**

Hospital / team: _____
Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

SACTCheck v0.84.45 · NCCP 00207 v6 · Wallet-card sheet
Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

CHAPTER 05

# Sources and further information

## IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.


1. **NCCP 00207 v5, page 1; current EMA Erbitux information**
   https://assets.hse.ie/media/documents/207_v5_Cetuximab_7day.pdf
2. **National Cancer Control Programme (NCCP) 00207 v6 · official regimen**
   https://healthservice.hse.ie/documents/6941/207_Cetuximab_7day_.pdf
3. **HSE: chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
4. **EMA: Erbitux product information and patient leaflet**
   https://www.ema.europa.eu/en/documents/product-information/erbitux-epar-product-information_en.pdf
5. **CO.17 KRAS analysis — primary report**
   https://pubmed.ncbi.nlm.nih.gov/18946061/
6. **NCCP emetogenic classification V6**
   https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_systemic anticancer therapy (SACT)_Induced_Nausea__pWgDCrm.pdf
7. **NCCP NCIS Medical Oncology antiemetics V8**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
8. **NCI: colon cancer treatment — carcinoembryonic antigen (CEA) and follow-up**
   https://www.cancer.gov/types/colorectal/hp/colon-treatment-pdq